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Please Provide the Following Information: Enthymemes and the Logics of Individual Responsibility in the Patient Medical History Form

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ABSTRACT

Despite the overwhelming evidence that health and risk are multifactorial, medical texts that interface with publics continue to circulate logics of individual responsibility of health. This article analyzes patient medical history forms across different times and spaces to track the logics of individual responsibility that operate in the form in various contexts. The analysis finds logics of individual responsibility are enthymized through genetic causation and personal choice and makes the case that texts, including mundane forms, perform patient education and construct patients' identifications with health, illness, and risk.

KEYWORDS

Enthymeme; risk; identity; patient education; forms; health literacy

In late 2021, over a dozen editors of leading medical journals around the world joined together to call on world leaders to take action against climate change to “halt the destruction of nature” as a way to protect planetary and public health (Atwoli et al., 2021, p. 602). These editors bring urgent attention to the growing impact of rising global temperatures on public health, including adverse effects on physiological and mental health, disease, food insecurity, and malnourishment, and the disproportionate effect rising temperatures have on the world's most vulnerable populations. In the US, to address the public health crises that are intensified by environmental factors, Healthy People 2030, an initiative led by the US Department of Health and Human Services (n.d.), which establishes nationwide public health objectives, lists Social Determinants of Health as one of five priority areas, including environmental conditions such as air and water quality, weather, and climate change, as these all can affect public and individual health. These institutions make clear that environmental degradation threatens human health.

Despite the immense influence that environmental factors, including climate change, have on public and individual health, medical forms that are in regular use by publics continue to circulate individual responsibility of health and ignore or minimize environmental, social, political, and geocultural factors. One such form is the Patient Medical History Form (PMHF), a widely used document in medical settings that snapshots a patient's personal and family health history. Drawing from posthuman theories of communication, I argue that the PMHF enthymizes logics of individual responsibility of health and risk for patient-users of the form. The PMHF presumes the medical provider (or medical team) as the default user and overlooks how patient-users use, interpret, or learn from the form's information. Because the medical provider and staff are perceived to be the intended users of the form, the questions are designed to efficiently and expediently gather information from the patient and to relay that information to the medical team. Without explaining reasons for the questions, the form relies on patient-users to “fill in” the unstated logics of the information it solicits,

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including what, exactly, is being asked and how that information matters in terms of the patient's perceived health and risk. Thus, the form flattens the many factors that affect health and risk.

To support this argument, I first review scholarship on the rhetorical and epistemological functions of forms and patient education materials. Then, to make the case that the PMHF mediates patient education, I describe posthuman theories of medical communication, specifically as represented by Gouge's (2017) work on the communication and knowledge as emergent in clinical settings and Johnson's (2018) concept of republications. I apply this framework to analyze PMHFs across different times and spaces to reveal how they communicate logics of individual responsibility about the medical subject to patient-users. By medical subject, I mean both the boundaries of medicine as a field and the subjectivity of the patient-user in the clinical encounter. In so doing, I demonstrate a need for health and medical communicators to treat *all* medical texts that patients use, including the PMHF, as performing patient education.

The PMHF is an important text to analyze for a few reasons. First, the text has many different users, including patients, patient advocates, medical providers, and clinic staff, all of whom approach or use the form with different experiences, knowledges, purposes, and priorities. Because medical providers are typically seen as the users of this text, the rhetorical work of the form on patient-users has mostly been ignored. Second, the PMHF is often the first point of contact between a patient and a new medical provider and therefore the first of many sites of doctor-patient communication. Consequently, the form orients the patient-user to logics of individual responsibility for health and wellness before the first meeting. Finally, as health initiatives continue to call for improving public health literacy and addressing health disparities, it is our responsibility as rhetoric and medical and technical communication experts to identify the mundane texts that might go overlooked, such as the PMHF, and analyze the role they play in health literacy and inequity.

The implications of this analysis extend beyond the PMHF. By considering the rhetorical and enthymematic work of forms, we can uncover how forms operate not only as tools that transmit information from one person to another but also as active agents in circulating institutional knowledges and discourses. When treated as facilitators of knowledge, and not only as vessels of information, forms can also be designed to more effectively and equitably educate their users and provide them with the larger social and political contexts in which they operate. In short, forms can contribute to users' critical awareness of their own situations.

Forms and functions, or the functions of forms

In professional and medical contexts, forms play an important, if often overlooked, role in advancing particular epistemologies and in shaping users' perceptions of institutional discourses. Indeed, scholars of technical and medical writing have done important work examining the cultural, ethical, informative, and persuasive work of forms, highlighting the many ways "forms work to funnel discourse through routinized and practical, institutional needs" (Keränen, 2014, p. 1174). In what follows, I highlight the rhetorical functions of forms to make the case that the PMHF is not merely a vessel that transmits information from patient to provider but actively shapes patient-users' knowledges of health, medicine, and patienthood.

The rhetorical work of mundane texts

Readers are likely well aware that professional, technical, and medical genres are commonly perceived to be true, objective, and neutral and, consequently, often escape critical rhetorical inquiry. Blythe (2007) calls these texts "mundane artifacts" because they "are not always as exciting or visible as the rhetorical fireworks" of other, more overtly persuasive documents (Rivers & Weber, 2018, p. 188). According to Blythe (2007), mundane texts play a crucial role in shaping institutions and people, and for this reason, they are important texts to analyze. Technical and medical communicators are often tasked with making texts efficient, though this "ethic of expediency" risks neglecting – or

exterminating – real human bodies (Katz, 1992). Echoing Katz, Balzhiser et al. (2019) remind us that “focusing primarily on functional aspects can leave us complacent, if not complicit, in systems of disenfranchisement, oppression, and discrimination” and that we have an obligation to pay attention to the ways forms enable or limit “respect, dignity, and human rights” (p. 4). Rhetoric and technical communication scholars, therefore, are uniquely positioned to analyze mundane texts not only for their effectiveness or efficiency but also for their ethical and cultural effects – that is, what these texts are *doing* to shape users’ knowledges and perspectives.

Indeed, mundane texts such as forms function by disciplining their users to adopt particular orientations of both content knowledge and ideology,¹ but because mundane texts are perceived to be neutral, objective texts, users do not often immediately recognize how or when they are being disciplined. Balzhiser et al. (2019) write, “Forms institutionalize discourses and influence thinking and behavior when people in authority endorse that information by using the form and passing it along to respondents” (p. 4). For instance, in her study of birth plans, Owens (2015) finds that when women write birth plans, they simultaneously “absorb beliefs and values” about childbirth (p. 43). Similarly, Seigel (2013) traces how pregnancy manuals not only teach users about pregnancy but also discipline women to perform pregnancy in certain ways while Koerber (2013) analyzes how breast-feeding discourses instill in breast-feeding mothers what counts as “normal” and “safe.” Keränen’s (2007) study of the patient preferences worksheet for end-of-life care reveals “how institutionally-inflected medical discourse structures decisions” about life and death (p. 181). Donovan’s (2015) analysis of the Application for Naturalization form reveals normalized racial logics that are present in federal forms and the marginalizing effect these logics have on the form’s diverse users. These analyses demonstrate the suasive work of forms on users and dispel the idea that forms merely transmit information from one person to another.

And yet, the everyday, mundane texts are where critical change and advocacy can happen in powerful ways. Rivers and Weber (2018) describe the 1955–1956 Montgomery Bus Boycott as a case study on the need for both mundane and monumental texts to sustain a movement. They claim that monumental texts such as Martin Luther King Jr.’s speech and Rosa Parks’ acts of resistance would not have been enough to accomplish the goals of the boycott – integration and civil rights – but were instead isolated rhetorical acts within a larger ecology of both mundane (e.g., newsletters, memos, legal documents, meeting minutes, etc.) and monumental (e.g., letters and speeches) texts. Neglecting the everyday, mundane texts ignores a significant body of works that work toward cultural and institutional change (Bordelon, 2012; Warren-Riley & Hurley, 2017). Indeed, powerful messages about the multifactorial contributors of health, illness, and wellness – such as environmental, political, social, personal, and genetic – must not be reserved for monumental texts like the editorial and public health objectives listed in the introduction. The texts that interface regularly with publics, like the PMHF, are in a position to sustain important messaging about complex factors of health and to invite users to reflect on and interrogate critically their health circumstances.

Informing and counterinforming through patient education materials

In their daily use, mundane texts such as forms are often treated as a rhetorical, technical, or legal documents that serve no rhetorical significance compared to texts that are more immediately rhetorically recognizable such as patient education materials (PEMs). Medical settings such as the clinic often provide patients with PEMs to provide vetted information about their visit or medical condition and include but are not limited to brochures, pamphlets, and discharge papers. Whereas forms are typically seen as unidirectional diffusions of information – for instance, the patient gives the medical staff their insurance information, and the medical staff gives the patient a copy of the patient privacy policy form – PEMs are designed with specific rhetorical goals to inform and persuade their users and to facilitate communication between patients and their healthcare team. PEMs, then, are part of health communication in that they are intended to give patients and their caretakers accessible, relevant, actionable information about their health.

PEMs are typically patient-centered in that they are designed to take into account patients' previous knowledge, education, concerns, fears, and goals and because they are created with patients and their families as the primary users. Therefore, PEMs are in a position to both educate and counter educate patient-users in light of what they already perceive to be true. For example, PEMs on antibiotics might anticipate the perception that medication should be taken only when feeling ill, such as taking an ibuprofen to treat a headache. Patients might incorrectly apply this same antecedent knowledge of when and how to take medication to antibiotics. (Medical experts advise patients to take all of their prescribed antibiotics, even if they begin feeling better sooner.) PEMs, therefore, can educate patients about their health conditions and counter educate misconceptions about health and wellness.

Professional medical organizations, patient advocacy groups, and public health institutions regularly call for improvements in patient education, such as making PEMs more accessible and understandable for general publics (Gouge, 2017; Renguette, 2016). These calls advocate for implementing universal design principles, translating materials for multilingual users, using short sentences, and avoiding jargon.² Although these efforts are necessary to make complex medical information more accessible to larger publics, narrow definitions of what counts as PEMs overlook opportunities and texts that, although not categorized as PEMs, still *perform* patient education. Ideas and beliefs about health and wellness are influenced by a wide range of discourses, technologies, and people who individuals interact with daily. Each interaction with medical discourses contributes to understandings of health, wellness, and risk. For instance, pharmaceutical commercials orient users to pay attention to phenomena (e.g., uncomfortable sensations in the leg) and understand them as symptoms of a health condition (e.g., restless leg syndrome). Such discourses contribute to publics' knowledge of health and wellness and can, therefore, be understood as *performing* patient education. By shifting our attention from texts traditionally classified as PEMs to include texts that *perform* patient education, we can more fully grasp the many texts and discourses that educate individuals, for better or worse, about health and wellness. The patient medical history form, I argue, is one such text that, although not classified as a conventional PEM, performs patient education through enthymematic reasoning.

Information sharing and identity formation in the patient medical history form

Also known as "patient history," "patient family history," and "patient intake," the patient medical history form (PMHF) is typically given to patients who are seeing a medical provider for the first time. The patient completes the form either on paper or electronically before or during the first visit with a new medical provider. Though each PMHF is different, they typically ask for demographic information, medications, previous surgeries, and health conditions. The triage nurse or medical assistant reviews the intake form and interviews the patient to clarify information and confirm the history. This information is entered into an electronic medical records database that the medical team can access. In later visits, the history is updated typically via verbal confirmation ("Has anything changed since your last visit?").

The patient's medical history is a crucial part of establishing medical care and is gathered for both preventive (such as a well-person visit) and curative (such as an ill-person visit) contexts. The form typically asks patient-users to provide information about health conditions that either they or members of their families have or have had, past surgeries, current medications, allergies, and social history. According to the American Medical Association (n.d.), the patient's individual and family health history are important to help the medical provider identify risk factors for disease, recommend treatment plans, indicate early warnings of disease, and develop a plan with the patient to adopt or maintain healthy lifestyle choices. In short, the primary purpose of the form is to transmit necessary health information from the patient to the medical provider.

Because the PMHF is one of the first tasks a patient completes when they establish care with a new physician, the form is in a unique position to establish a patient-provider relationship. The form establishes this relationship by facilitating communication between patient and physician and by constructing the particular roles that the patient and physician perform. Bawarshi (2001) claims the

PMHF enables identity formation based on how it is used by both patient and physician. The physician's office and PMHF work together in the same rhetorical ecosystem to shape social interactions, putting the patient in the patient role and physician in the physician role. The PMHF "helps us assume certain social roles, roles established by our culture and rhetorically reproduced and enacted by the genre, which in turn help us perform certain activities in certain ways" (Bawarshi, 2001, p. 74). Traditionally, these roles include who gives information to whom: patients give information about their medical history to the medical provider, and the medical provider gives information to patients about their medical conditions. In this way, the PMHF, as a genre, not only transmits information from one person to another but also actively shapes the social roles between patient (the body acted upon) and provider (the expert who acts upon the patient). Or, as Popham (2005) says, the form positions the physician as scientist and the patient as scientific data.

Together, the medical scholarship and Bawarshi's work on the PMHF demonstrate how this mundane text actively communicates and orients bodies in the clinical encounter. The medical literature shows the PMHF is an important tool that communicates medical knowledge to the medical provider, and Bawarshi demonstrates the social action and identity formation that the PMHF performs. Not only does the form communicate medical history information from the patient to the doctor, but it also, I argue, generates meaning about health, wellness, and risk for the patient-user.

Emerging and circulating knowledge through the enthymeme

Posthuman theories of medical and technical communication advance the idea that communication is not an unidirectional transaction between speaker and listener; rather, it emerges in complex assemblages of bodies and contexts. As Mara and Hawk (2010) claim in their introduction to the special issue of *Technical Communication Quarterly* on Posthuman Rhetorics and Technical Communication, "Attempts to understand human beings as autonomous, isolated, or determinative do not fully account for the complexities of living, writing, and working in a variety of biological and mechanical systems" (p. 2). In her work on doctor-patient communication, Gouge (2018) argues that knowledge is not merely transacted from one person to the other but is also "formative, multi-directional processes of negotiation and exchange" (p. 6). In the biologics of the clinical setting, she explains, knowledge is not simply transmitted from doctor to patient or from patient to doctor; rather, the clinical encounter generates knowledge for both.

The knowledge that emerges in the clinical encounter is part of a larger network of biomedical logics that become "republished" in various encounters. As Johnson (2018) explains, republications circulate and amplify certain discourses to make those discourses standardized information infrastructures. He claims:

Republication bombards public discourse by repeating the same information in the same format over and over again across times, spaces, and people, lending credibility to a knowledge regime. As such, republications provide the background work enabling knowledge-making about health and medicine. They continually compel audiences to their legitimacy because they are so prevalent. (p. 75)

Johnson (2018) uses the nutrition label to exemplify republications. The nutrition label foregrounds specific nutrition information, such as protein. As a result, "The standardized food label generates a regime of health infrastructure that continually reminds the public of one notion of health" (p. 74).

Logics of individual responsibility of health are one such information infrastructure that circulates a particular understanding of health and risk. As my analysis shows, logics of individual responsibility of health are framed in two primary ways: health as genetic causation and as personal choice. In the age of biomedical neoliberalism (Leach, 2020), we are bombarded with such messaging of individual responsibility. For instance, genetic testing kits may tell users the diseases they are at risk of inheriting, and BRCA testing has become widely practiced because of the genetic rhetoric that has stuck to the mutation. Similarly, efforts to persuade publics to adopt healthier lifestyles encourage them to make "better choices" about their wellness. In this way, beliefs about health as genetic causation and as personal choices function as republications in that they locate responsibility for health, illness, and risk within the individual body.

When extended to the PMHF, genetic causation and personal choices of health are republished through enthymemes that circulate this singular understanding of health and risk as individual responsibility.

Although genetic disorders such as sickle cell anemia, cystic fibrosis, and Tay Sachs disease are directly related to genetics (National Human Genome Research Institute, 2018), health, disease, and risk are not typically predetermined by genetics but are the result of complex interactions among genes, lifestyle, and environment in what are called complex or multifactorial disorders. Indeed, “both zip codes and genetic codes are important for our health” because “most human disease results from the interaction of our genetic susceptibility with environmental and behavioral risk factors, such as diet, physical activity, infectious agents and the physical environment” (Khoury, 2014). Therefore, circulating individual logics of health deflects attention from environmental, political, industrial, social, and geocultural contributors to disease and risk. As Caitlin Leach (2020) notes, practitioners often note the social determinants of health, but “the biomedical guidelines of evidence-based medicine often fail to address these determinants” (p. 144). In other words, despite widespread recognition that health, illness, and risk are multivariant, in practice, evidence-based medicine prescribes logics of individual responsibility of the body and health. It is through this logic, then, that I read the enthymematic inferences of PMHFs. When recast as performing patient education, the PMHF is in a position to present this more robust understanding of health and risk as affected by environmental, geocultural, political, and social factors.

Although the PMHF isn’t typically classified as a patient education material, I argue that the form *performs* patient education enthymematically. Enthymemes are “informal and incomplete reasoning patterns; they are informal because they are based on popular opinion/cultural values and incomplete because a premise is left out of the statement” (Newman, 2009; refer also to, p. 281; Ding, 2014; Koerber, 2018; Scott, 2002; Teston, 2017). Recent scholarship resists conventional definitions of the enthymeme as a deductive argument composed of a major premise, a minor premise, and a conclusion, with one of these parts missing. Instead, enthymemes rely on inference, context, and situated knowledge to construct an implicit argument (Fredal, 2020). As such, enthymemes require the audience to “fill in” the missing logics or the implicit components of the argument. In this way, the audience “participate[s] in the construction of the argument” and in their own persuasion (Newman, 2009, p. 281). In the PMHF, the logics of the requested information are not clearly stated and rely on patient-users to “fill in” the missing logics of the form to shape how they understand the relationship between personal health, family history, and risk.

By requiring the audience to fill in the missing logics, enthymemes can be both deductive and inductive. According to James Fredal (2020), “some enthymemes reason not from universals but from particulars” (p. 69). Indeed, as my analysis shows, the PMHF works both deductively and inductively. Through deduction, patient-users glean the requested information on the form within the context of the biomedical clinic to conclude their own assessments of how health, illness, and risk operate through individual and genetic causation. At the same time, the PMHF works inductively by considering the particulars of the form and drawing larger conclusions or ideas about boundaries and commitments of the medical institution writ large. The enthymeme, therefore, plays an important role in “how we think, communicate, and learn” (Newman, 2009, p. 291).

Though commonly presumed to be verbal arguments, recent scholarship on enthymemes demonstrates how enthymemes can also be visual (Finnegan, 2001), gestural (Newman, 2009), and embodied (Prenosil, 2012). In fact, Fredal (2020) locates enthymemes as part of our lived spaces, claiming that “human reasoning [is] based less on formal manipulation of propositions than on experience with social worlds” (p. 181). Importantly, enthymeme’s conclusions “do not just exist” but “*emerge* as the consequence of – and always in relation to – a series of premises, a history and field of other conclusions” (Sharp-Hoskins, 2023, p. 7). When extended to the PMHF, this expanded take on the enthymeme captures the complex ways the clinical context in which the form appears, the information the form solicits, and the patient-users’ embodied experiences and knowledges aid patient-users in filling in the logics of health and risk within the backdrop of the medical information infrastructure. The PMHF, I argue, locates responsibility of health and risk in individual bodies, such as through genetic causation and personal behaviors and lifestyle. Consequently, the

form deflects attention to the collective and distributed responsibility of health and risk, such as the environmental, geocultural, social, and political factors that influence health, wellness, and risk.

To examine how the PMHF performs patient education enthymematically, I first track the historical developments that led to the emergence of the PMHF. As I will show, the PMHF emerged as a way for physicians to gather information from the patient, which consequently positioned the physician as primary user of the form and obscured the effects of the form on the patient-user. Next, based on my analysis of nearly three dozen PMHFs from medical clinics in the United States, I surface the unstated premises that are implicit within and across the genre. These forms represent a variety of medical practices, including general practice, cardiology, sports medicine, women's health, cardiovascular, and ear, nose, and throat specialties. I draw from a range of medical contexts to track enthymematic patterns of the PMHF; specifically, I consider what logics of health, wellness, and risk are implicit and republished in these forms. Finally, I look at how the PMHF is used beyond preventive and curative medicine, such as in applications for life insurance coverage, immigration, and military enlistment. Like in medical contexts, these enactments of the form locate responsibility of health, illness, and risk on the individual body, but they do so to assess the risk these individual bodies pose to others. By analyzing the PMHF across times, places, and contexts, I examine the enthymematic republications of infrastructural logics of individual responsibility of health, illness, and risk. I conclude with ideas on how the PMHF might be reimagined so that the logics it provides to the patient-user are predicated on a distributed responsibility of health, illness, and risk.

The emergence of the PMHF

Taking a patient's medical history has been an important part of medical care for centuries. Before preventive medicine emerged as a formalized medical practice in the twentieth century, the physician's role was primarily to diagnose and treat ill patients. The patient's medical history, then, consisted of what is now called the history of the present illness³ to give physicians clues to help them diagnose illness. The physician gathers clues as to the cause of illness through the patient's narrative of their illness, through learning about the patient's habits such as sleep and diet, and through understanding the patient's natural disposition to know what is normal for the individual. In ancient Greece, Rufus of Ephesus also believed the manner in which patients respond to questions – such as the strength of their voice, focus, mental clarity, and lethargy – could also offer clues. For him, knowing the patient's illness, habits, and natural disposition “can only be known by asking, and to know them is, in practice, more important than anything else” (Brock, 1972, p. 116). Similarly, Galen and Hippocrates considered the effects of the patient's home and community environments on health. Galen would examine patients in their own environments to better understand the factors that might be contributing to illness. Hippocrates accounted for the effects of the seasons and air and water quality on patients' health. For them, a patient's medical history must be contextualized within their lived environments (Nutton, 1993).

The patient's medical history, not the physical exam, often serves as the sole source in diagnosis. Before the physical exam emerged as a common practice in the Victorian era in England, physicians often relied on the patient's narrative to diagnose and treat illness (Porter, 1993).⁴ The patient would “tell the doctor what was wrong: when and how the complaint had started, what events had precipitated it, the characteristic pains and symptoms, and its periodicity. The patient would also describe to his physician key lifestyle features – his eating and sleeping habits, his bowel motions, recent emotional traumas and so forth” (Porter, 1993, pp. 181–182). Through repeated experience of narrating their illnesses, patients learn what to pay attention to in order to share their history. Even today, the history of the present illness can often provide the necessary information to the provider to make a diagnosis.⁵ Medical internist and writer Ofri (2017) claims, “For most medical ailments, nearly all the crucial diagnostic information is gathered in the history, in the conversation between doctor and patient. The physical exam serves mainly to confirm the diagnosis” (p. 150).

The patient's medical history would serve new purposes with the emergence of preventive medicine in the early- to mid-twentieth century. In curative medicine, the patient's history, or the history of the present illness, aided the physician in making a diagnosis of disease. In preventive medicine, when a patient visited a physician for ongoing wellness rather than to seek treatment for illness, the patient's history provided information to the physician to indicate markers of risk for potential illness. The medical history as assembled in the PMHF as an established, recognizable form emerged along with preventive medicine to document these markers. In 1923, the American Medical Association endorsed the practice of periodic health exams for preventive health care and passed a resolution to "prepare forms suitable for use by practitioners of medicine in carrying out the purposes of the periodic health examination" (Emerson, 1923, p. 1376). In a special article published in the *Journal of the American Medical Association (JAMA)*, Emerson (1923) presents a PMHF that consists of two forms: Form A documents the patient's medical history, and Form B is where the physician documents their observations from the physical exam and notes the advice given to patients. Emerson (1923) suggests patients complete the form before their scheduled appointment so "the physician's time can be materially saved"⁶ (p. 1377). At the appointment, the physician can verify information and ask follow-up questions as needed.

Emerson (1923) suggests the physician is the primary user of the form. The primary goal of the PMHF, he acknowledges, is to provide background information on the patient to the physician. The form compels physician-users to keep methodical notes of their observations, which would allow them to "acquire an increasing skill and keenness in detecting early evidences of preventable and curable conditions" (p. 1376). For instance, the form includes questions such as "Do you support yourself?" and "What are your home conditions?" that Emerson recognizes "are not commonly sought by physicians in taking personal histories of the sick," (p. 1377) but provide insight into the patient's economic and social environment. These insights are important because, he says, "Functional nervous disorders are not uncommonly explained chiefly by the anxiety and distress which result from narrow resources of money and friends" (p. 1378).

Yet, Emerson (1923) suggests that the form also performs important enthymematic functions for the patient-user as well. By completing the forms, patients "will learn to appreciate the importance to their own health of good medical examinations, and to expect attention to the details of personal hygiene on which continued health often depends" (p. 1376). He acknowledges that completing the form can affect patients' thinking and behaviors, even if the responses the questions generate are not fully useful for the physician. For instance, questions on the patient's food and drink consumption and alcohol and tobacco use "will suggest to the mind of many patients the necessity of some thought and attention to the elements and proportions of their food and drink" (Emerson, 1923, p. 1378). In other words, by simply raising the question, the PMHF could instigate patient-users to take pause and perhaps pay more attention to what they are consuming and how their consuming practices might affect their health.

Over the next few decades, other PMHFs will emerge with the primary goal of making history taking more efficient for the physician. In 1949, the Cornell Medical Index (CMI), a 195-question form that became widely used across the US, was developed as a "quick and reliable method of obtaining important facts about a patient's medical history without expenditure of the physician's time" (Brodman et al., 1949, p. 530). To gather such quantities of data that physicians can review quickly, the form is organized with questions in the left column and yes/no options in the right. This way, the physician can glean the responses and look for any answer that has been circled "yes" and ignore negative histories. Additionally, Brodman et al. (1949) promote the CMI for its "standardization of the data" that can be used for research, teaching, and comparisons among patients (p. 534). In this way, both the content and the structure of the PMHF are framed in terms of ease of use for the physician-user.

As the PMHF became more widely used, medical researchers and providers worked to ensure their forms were tested for validity (that the questions provided insightful information for the medical provider) and reliability (that patient-users provided consistent, accurate responses). The CMI, for

instance, urges physicians to use the form because patients answer “as honestly and accurately as they do similar questions in interview” (Brodman et al., 1949, p. 531). Similarly, a team at the Seattle Veterans Administration Medical Center created its own PMHF, tested it for validity and reliability, and promoted the fact that it “requires no assistance to the patient during completion and no transcription or review before use by the health care provider” (Pecoraro et al., 1979, p. 231). The team’s goal was to limit the amount of time staff spent on new-patient tasks, such as collecting medical history, to allot more time for patient communication and education. By the 21st century, studies in medical literature have accepted the importance of the PMHF and focus more specifically on approaches that yield the most accurate information for the medical team in the most efficient way, such as through written questionnaire, dialogue interview, or computer interface (Boissonnault & Badke, 2005; Burnham et al., 2002; Dominguez et al., 2007; Hershey & Grant, 2002; McDaniel et al., 1995; St Sauver et al., 2005).

Medical technologies that track forms, including the PMHF, have been around since at least the 1960s but became more widely used in the 21st century with the push toward electronic medical records. Medical records technologies hold the promise of efficiency and satisfaction for patients, physicians, administrators, and insurance companies (Bachman, 2007; Bouamrane et al., 2008; Fernando et al., 2012; Melms et al., 2021). Electronic medical records work to standardize patients’ health histories to make them recognizable to others, such as those who may request these records to transfer medical care or to assess eligibility for insurance coverage. Relatedly, medical records apps claim to streamline the intake process by allowing the patient to complete the PMHF before the visit and to make records sharing easier and more secure. Some programs even claim to “translate” the patient’s medical history into clinical terminology for the physician, shaving off minutes from each clinical visit. These programs market themselves based on how easily the patient-user can complete the form, how efficiently the form conveys the patient’s information to the provider, and how easily (and securely) the records can be shared with others.

Since the mid-20th century, the messaging around the PMHF cohered around its central function as an aid to the medical provider in making diagnoses and creating treatment plans for the patient (Fawcett & Rhynas, 2012; Summerton & Garrood, 1997). These treatment plans tend to focus on modifying individual behaviors or taking precautions in light of genetic risk. In fact, according to the American Medical Association (n.d.), one of the key functions of the family health history is to “help plan lifestyle changes to keep the patient well.” The information the PMHF and physical exam provide the physician prepare the physician to advise the patient, but, as Emerson claimed in 1923, medical counsel ought to consist only of actions the individual patient can do to their own body:

The advice given should consist of written directions to the patient, for the correction or limitation of defects or errors discovered. Simple, precise, nontechnical wording should be used, and *only such measures should be proposed as fall within the possibilities of the patient to carry out.* (p. 1379, emphasis added)

Put another way, even though health, wellness, disease, and risk are constituted by several factors – genetic, environmental, cultural, political, geographical – medical advice targets the individual body. Limiting the medical information and advice at the individual level also limits patients’ understandings of the multiple factors that affect health and wellness. In short, the patient is left with an individualistic understanding of their own body. The PMHF, then, is crafted in a way that compels the physician-user to gather limited information about and intervene on the patient’s individual body. In this way, taking a patient’s history is what Marya and Patel (2021) call a “selective act” that recognizes only limited contributors of health and illness, or “what medicine had already decided was relevant” (p. 331). Factors such as the larger and longer effects of neoliberal, colonial, industrial, racist institutions aren’t recognized in the PMHF because they are perceived to be measures that fall beyond “the possibilities of the patient to carry out.”

In the nearly hundred years since the *JAMA* first presented a PMHF for wide use, it’s clear that the physician has emerged as the default user of the form. The information gathered from the patient allows the physician to assemble a diagnosis and action plan that targets the patient’s individual body.

When patients have been considered as users, the focus has been primarily on assessing their ability to complete the form unassisted, both in terms of physical and cognitive ability and comprehension, and to provide reliable and valid information to the presumed user, the physician. To be clear, these studies are necessary to ensure forms can be used by diverse patient users, regardless of age, gender, language, nationality, ability, or technical expertise. Still, what remain ignored are the disciplinary effects the PMHF has on the patient-user.

The PMHF's enthymemes

The previous section traced the emergence of the PMHF and how the physician became the presumed default user. In what follows, I consider the patient as user and describe the patterns of the PMHF genre that circulate knowledges of the medical subject, both medical subject content and medical subjectivity, through logics of individual responsibility. I am not advocating for one type of form over another, but am taking a closer look at what the forms are doing enthymematically to circulate logics of individual responsibility of health and risk.

Establishing medicine's boundaries

The PMHF enthymematically establishes the boundaries of medicine and disease that matters in biomedical spaces. It does so through organizational schema that require inductive and/or deductive reasoning about what counts as medical topics and as symptoms in specific medical contexts. Through inductive reasoning, the patient-user can glean the information the form is requesting to draw larger conclusions about what counts as medical matters. Several forms I analyzed include lists of specific medical conditions and ask the patient-user to indicate whether they have and/or a family member has had a history with any of those conditions. Some conditions were present in nearly all the forms, such as heart disease, cancer, high cholesterol, and high blood pressure (Figure 1). Through republication, listing specific medical conditions codifies what counts as a medical topic. For instance, naming health conditions like depression, arthritis, and alcoholism teaches patients that these conditions are considered medical conditions. Listing specific conditions also helps to trigger patients' memories of their families' health histories. A patient-user might not recall a family member's heart disease diagnosis, for instance, but might remember by seeing it listed on the form. Yet, a list of medical conditions can also suggest a definitive catalog of the medical topics that matter in the clinic and closes off other potential medical topics for the patient. Through inductive reasoning, the patient-user draws from these specific conditions to develop an understanding of what counts as a medical topic.

Similarly, the PMHF codifies the boundaries of what counts as a symptom in medical contexts through inductive reasoning. This codification happens when forms list symptoms and ask the patient-user to indicate which, if any, they are experiencing. For example, a form from a cardiovascular clinic lists symptoms (such as weight loss, passing out episodes, and headaches) and asks the patient-user to indicate whether they are experiencing the symptom by circling yes or no (Figure 2). Without clearly stating the logical connection – how the symptoms listed are related to heart disease or other heart conditions – the form enthymematically teaches the patient what counts as a symptom as it relates to cardiovascular health and, in so doing, teaches the patient what to pay attention to in their own bodies. Similarly, some PMHFs organize symptoms inductively by organ system. In this approach, the form identifies the body systems (such as Constitutional, Cardiovascular, Musculoskeletal, etc.) and lists common symptoms associated with each system. In one example form from a primary care facility that uses this structure, under the Constitutional system heading, symptoms listed are fever, night sweats, weight gain, weight loss, and exercise intolerance (Figure 3). This organization of systems and symptoms offers a mini lesson of sorts to the patient-user about the body's different body systems and common symptoms that are associated with each. Through inductive reasoning, then, the patient-user momentarily sees the body through a medical perspective – through organ systems.

Family and Self History	Self	Family
Heart Attacks	☐	☐
Serious Heart Trouble	☐	☐
High Blood Pressure	☐	☐
Strokes	☐	☐
Leukemia	☐	☐
Other Cancer	☐	☐
Diabetes	☐	☐
Thyroid Disease	☐	☐
Seizures	☐	☐
Migraines	☐	☐
High Cholesterol	☐	☐
Asthma	☐	☐
Alcoholism	☐	☐
Anemia	☐	☐
Bleeding Problem	☐	☐
Cataracts	☐	☐
Mental/Nervous Disorder	☐	☐
Tuberculosis (or exposure)	☐	☐
Blood Clot	☐	☐
Emphysema/Chronic Bronchitis	☐	☐
Stomach Ulcer/Gastritis	☐	☐
Gallbladder Disease	☐	☐
Hepatitis	☐	☐
Cirrhosis/Liver Disease	☐	☐
Bowel/Colon Disease	☐	☐
Hemorrhoids/Rectal Problem	☐	☐
Hernia	☐	☐
Kidney Disease/Infection	☐	☐
Gonorrhea or Syphilis	☐	☐
Arthritis	☐	☐
Bone or Joint Disease	☐	☐
Rheumatic Fever	☐	☐
Polio	☐	☐
Meningitis	☐	☐
Glaucoma	☐	☐
Hay Fever/Allergic Rhinitis	☐	☐
Hives or Eczema	☐	☐
Abnormal Pap or Breast Mass	☐	☐
Other Disease	☐	☐
Explain		

Figure 1. A list of medical conditions. The PMHF often lists medical conditions and asks the patient-user to check the box if they or a member of their family have ever had the condition.

Another way the PMHF establishes the boundaries of medicine and disease that matters in biomedical spaces is through deductive reasoning. Rather than listing medical conditions, some PMHFs include space for the patient-user to list the different conditions, diseases, and disorders their family members have had. One form from a primary care facility provides space for the patient-user to write in the ages and “significant health problems” of their father, mother, siblings, children, and grandparents (Figure 4). Identifying health problems relies on antecedent knowledge of the medical subject for the patient-user to know what to include and what not to include based on their understanding of the boundaries of what counts as a medical problem.

DO YOU HAVE ANY OF THESE SYMPTOMS? (circle answer)

Weight Loss	Yes No	Passing out episodes	Yes No	Headaches	Yes No
Palpitations or rapid beating heart	Yes No	Changes in bowel habits	Yes No	Seizures	Yes No
Shortness of breathe at rest	Yes No	Nausae/vomiting/ diarrhea	Yes No	Temporary blindness in eye	Yes No
Unusual shortness of breathe on exertion	Yes No	Vomiting blood or blood in your bowel movements	Yes No	Numbness in arm or leg	Yes No
Difficuly breathing At night	Yes No	Painful urination	Yes No	Leg pain/fatigue with walking	Yes No
Chronic cough	Yes No	Excessive bleeding or Easy bruising	Yes No	Abdominal pain	Yes No
Chest pain or pressure at rest	Yes No	Chest pain or pressure exertion	Yes No	Anxiety	Yes No

Figure 2. A list of symptoms. The PMHF often asks the patient-user to indicate if they are experiencing symptoms from a provided list.

REVIEW OF SYSTEMS

Constitutional: ☐ Fever ☐ Night sweats ☐ Weight gain ☐ Weight loss ☐ Exercise intolerance	Eyes: ☐ Dry eyes ☐ Vision change ☐ Irritation	Ears and Nose: ☐ Difficulty hearing ☐ Ear pain ☐ Frequent nosebleeds ☐ Nose problems ☐ Sinus problems	Mouth and Throat: ☐ Sore throat ☐ Bleeding gums ☐ Snoring ☐ Dry mouth ☐ Mouth ulcer ☐ Oral abnormalities ☐ Teeth problems
Cardiovascular: ☐ Chest pain ☐ Arm pain on exertion ☐ Shortness of breath when walking ☐ Shortness of breath when lying down ☐ Palpitations ☐ Known heart murmur	Respiratory: ☐ Cough ☐ Wheezing ☐ Shortness of breath ☐ Coughing up blood ☐ Sleep apnea	Gastrointestinal: ☐ Abdominal pain ☐ Vomiting ☐ Change in appetite ☐ Diarrhea ☐ Vomiting blood ☐ Indigestion ☐ GERD	Genitourinary: ☐ Incontinence ☐ Difficulty urinating ☐ Blood in urine ☐ Increased frequency
Musculoskeletal: ☐ Muscle aches ☐ Muscle weakness ☐ Joint pain ☐ Back pain ☐ Swelling in extremities	Integumentary: ☐ Abnormal mole ☐ Jaundice ☐ Rash ☐ Laceration	Neurologic: ☐ Loss of consciousness ☐ Weakness ☐ Numbness ☐ Seizures ☐ Dizziness ☐ Migraines ☐ Headaches ☐ Tremors	Psychiatric: ☐ Depression ☐ Sleep disturbances ☐ Unsafe relationship ☐ Alcohol abuse ☐ Anxiety ☐ Hallucinations ☐ Suicidal thoughts
Endocrine: ☐ Fatigue	Hematologic/Lymphatic: ☐ Swollen glands ☐ Bruising ☐ Excessive bleeding	Allergic/Immunologic: ☐ Runny nose ☐ Sinus pressure ☐ Itching ☐ Hives ☐ Frequent Sneezing	Other/not listed:

Figure 3. A table of body systems with corresponding symptoms.

This more open-ended approach to gathering a patient’s history also allows the patient-user to decide what counts as a “significant” health problem that is worth listing on the form and to identify conditions that are not necessarily or traditionally medical subjects but concerns that matter to them or to their families. For example, patient-users might decide that a family history

FAMILY HEALTH HISTORY							
AGE		SIGNIFICANT HEALTH PROBLEMS		AGE		SIGNIFICANT HEALTH PROBLEMS	
Father				Children	☐ M		
					☐ F		
Mother					☐ M		
					☐ F		
Sibling	☐ M				☐ M		
	☐ F				☐ F		
	☐ M				☐ M		
	☐ F				☐ F		
	☐ M			Grandmother			
	☐ F			Maternal			
	☐ M			Grandfather			
	☐ F			Maternal			
	☐ M			Grandmother			
	☐ F			Paternal			
	☐ M			Grandfather			
	☐ F			Paternal			

Figure 4. Open text family history.

of breast cancer is important to list but might decide that asthma is not a significant health problem.

The PMHF also codifies what counts as a symptom deductively through open-ended response. In this approach, the form asks the patient-user to list the symptoms they are experiencing rather than select from a predefined list. This approach allows the patient-user to write down the symptoms they are experiencing that may not be identified in other parts of the form. This approach gives the patient-user an opportunity to communicate what they are experiencing and requires them to make decisions about what counts and does not count as a symptom. On one form from an otolaryngology clinic, the form asks the patient-user, “What problems are you having?” and provides three lines to list each complaint (Figure 5). By numbering the complaints, the form encourages the patient-user to prioritize the most pressing symptoms or concerns, which then will, presumably, structure the consultation itself.

The context of these forms will also certainly influence how the patient-user makes decisions about what to include and what to exclude. For instance, if completing the form at a cardiology clinic, the patient-user might recognize issues like family history of heart disease or heart attack as relevant but might not think to name diabetes or other health conditions that, for non-expert patient-users, might not have an obvious connection to the heart. Through deductive reasoning, the patient-user draws from their own understandings of what counts as medical topics, or in this case, what counts as health conditions, to decide which conditions to list on the form.

Because the PMHF helps establish the boundaries of what counts as a medical topic, it is also in a position to introduce new(er) medical topics to the patient-user, especially regarding topics that are not clearly or historically medical. Recently, PMHFs have begun asking about mental health and patient safety, which historically have been beyond the boundaries of medicine. One form from a women’s cardiovascular health center screens for depression by asking two questions about the patient’s “recent mood” (Figure 6). Several forms ask about intimate partner violence (IPV) either directly or indirectly, which is a relatively new question

Medical Information

What problems are you having? (List each complaint, how and/or when it first started)

1. _____

2. _____

3. _____

Figure 5. Medical problems list. Some forms ask the patient-user to list the medical concerns they want to address in the visit.

Please answer the following two questions about your recent mood.

	Yes	No
1. In the past month have you often been bothered by feeling down, depressed, or hopeless?	☐	☐
2. During the past month, have you often been bothered by little interest or pleasure in doing things?	☐	☐

Figure 6. A depression screening.

Trauma History: Have you ever been the victim of trauma or abuse (including sexual, emotional, physical abuse or neglect and/or being a victim of an accident, violent crime, or a natural disaster)? Yes No

If yes, is this an active issue in your life that you would like to address while you are here? Yes No

Figure 7. A trauma screening.

to ask in the medical space. A number of medical organizations advocate for screening patients for intimate partner violence, and one way to do this is by including a question on the PMHF. The American College of Obstetricians and Gynecologists (2012) recommends that medical providers “incorporate screening for IPV into the routine medical history by integrating questions into intake forms so that all patients are screened whether or not abuse is suspected” (p. 3). Similarly, a form from an integrative health practice asks patients about trauma more generally, such as if the patient is a victim of abuse or neglect or of an accident or natural disaster (Figure 7). Other questions in similar “patient safety” sections across PMHFs also ask whether the patient uses their seatbelt in the car, wears sunscreen, has allergies to latex and insect stings, or lives alone.

By making space for these topics on the PMHF, the medical field opens up the boundaries of what counts as a medical topic, and the patient-user learns that relevant medical topics also include their physical safety and emotional and mental health. These boundaries adjust over time as different healthcare needs become more and less pressing; in other words, the questions the forms ask can be adapted to respond to some of the most current, pressing health issues. For instance, though only one of the PMHFs I analyzed asked whether the patient-user keeps a gun in the home, a growing movement in leading medical organizations seeks to reduce injury and death due to firearm violence. To address firearm violence, the medical field identifies it as a public health crisis (Butkus et al., 2018). In fact, some clinicians are addressing gun safety in their medical practices (Pallin et al., 2019). They do so by identifying patients who are most at risk of gun violence by high-risk demographic groups, alcohol and drug use, intimate partner violence, or poorly controlled mental illness. Most of this information can be gathered through the PMHF.⁷

Opening up these boundaries is not without potential issues. On one hand, expanding the boundaries of what counts as a medical topic risks medicalization creep. Many valid arguments have been made about the overmedicalization of the body, and expanding what counts as a medical issue might lead to unintended and unfortunate consequences ranging from intrusive medical oversurveillance to even more rushed medical visits.⁸ On the other hand, expanding what counts as a medical topic can help legitimize and destigmatize real health threats, such as mental health; account for the intersecting networks that affect health, such as environmental degradation and racism; and address the most pressing health issues in real time. This expansion speaks to the

larger social and political influence of the medical establishment to bring awareness and resources to current health issues.

Risk as a/of the medical subject

A central component of the PMHF is gathering both the patient's health history and the histories of their family members. The medical provider uses this information to identify whether a patient is at increased risk for disease and to develop a care plan for (and with) the patient. For patient-users, the PMHF enthymematically circulates logics of individual responsibility, specifically through genetic causation, by defining family as blood relatives. For instance, nearly all forms ask the patient to indicate medical conditions that the patient and/or members of the patient's family have experienced. Some forms explicitly ask for health histories of "blood relatives" only (Figure 8). Other forms are more implicit with genetic logics of risk. In one form from a general practice office, specific family members are listed, including maternal and paternal grandmother and grandfather (Figure 4). By defining family as biological or genetic, the PMHF risks conflating causation and correlation of risk. For instance, asthma can be caused by both genetics and environment, but through logics of individual responsibility, the enthymeme presumes genetic causation. However, in many cases, the relationship between health and risk can be correlational. For example, Type 1 diabetes can have genetic causes, but Type 2 diabetes is often correlational. By defining family as blood relatives, the PMHF highlights risk as causative (through genetics) and obscures risk as correlational (e.g., environmental, geocultural, political, social, behavioral).

Presenting family as genetic or blood relatives reflects normative, limited conceptions of family and suggests that health risk is genetically determined. Adopted family members, blended families, and those who were conceived via egg or sperm donor may not know their complete "blood relative" medical history. One form I analyzed gave the patient-user the option to select "Adopted/Don't Know" in the family medical history section. However, by limiting family history to only blood relatives, the PMHF deflects nongenetic factors that play a significant role in health and risk. In fact, knowing the health histories of nonbiological family members, especially those who share a household, is just as important as knowing health histories of blood relatives. According to the Centers for Disease Control and Prevention (2022), families often share similar health risks not only for genetic reasons but also because of lifestyle: "You may also have behaviors in

Family History		
Has a blood relative ever had	Y	N
Anemia		
Arthritis		
Diabetes		
Glaucoma		
Heart Disease		
High Blood Pressure		
High Cholesterol		
Thyroid Disease		

Figure 8. Blood relative family history list. Some PMHFs ask specifically about blood relatives' medical history.

Have you ever been exposed to:		
The AIDS virus (HIV)	☐ yes	☐ no
Tuberculosis (TB)	☐ yes	☐ no
Hepatitis virus (A, B or C)?	☐ yes	☐ no

Figure 9. A list of viruses. Some PMHFs ask if the patient-user has been exposed to viruses.

common, such as exercise habits and what you like to eat. You may live in the same area and come into contact with similar things in the environment. Family history includes all these factors, any of which can affect your health.” Yet, through logics of individual responsibility, the PMHF teaches the patient-user that risk of disease is limited to genetics.

Logics of individual responsibility also materialize through personal choice. In this case, risk is often presented as something knowable and certain, particularly when it comes to exposure to certain contagions. For instance, a form from a general practice clinic asks the patient-user if they have been exposed to certain viruses, specifically HIV, tuberculosis, and hepatitis A, B, or C, and provides “yes” and “no” as options to check (Figure 9). Requesting this information in this way suggests people know with certainty if they have been exposed to a virus. Although it is not uncommon for people to know if they *have* been exposed to a virus, it is nearly impossible to know if they *have not* been exposed to a virus. Consequently, asking patient-users to respond with either yes or no might give false reassurance that they would know if they have been exposed (Refer to Scott, 2003).⁹

By enthymematically framing risk as individual responsibility through genetic causation and personal choices, the PMHF positions health and risk as located solely in the individual body (refer also to Mara, 2010). This sentiment echoes biomedical information infrastructures that position health and risk as an individual responsibility rather than a distributed one. Medical advice, then, might include behavioral changes to diet and exercise, stress reduction, medication, and other actions that individuals can modify to their own bodies and obscures other complicated factors that affect health and risk. Consequently, the PMHF leaves no room for patient-users to interrogate these other confounding, correlational factors of health and risk, such as local pollution, quality and safety of public water system, availability of healthy foods (food deserts), and other factors that influence health and risk that are beyond an individual’s ability to control. Pender (2018) claims similar messaging is prevalent in breast cancer prevention discourse. As she claims, women are instructed:

To prevent breast cancer through smart lifestyle choices or to catch it early through vigilant surveillance have been promulgated more persistently and more loudly than any other, creating a situation in which individual women—not the companies that pollute, the manufacturers that make carcinogenic products, or the politicians who fail to improve people’s economic and environmental conditions—are responsible for protecting themselves from this disease. (p. 32)

Changing the narrative of health and risk, even through ordinary, mundane texts such as the PMHF, can provide patient-users with more accurate information and context on the wide scope of factors that constitute risk.

The medical periphery of social history

As I have been arguing thus far, the PMHF circulates logics of individual responsibility of health and wellness by enthymizing health and risk as genetic causation and as personal choices. The latter is especially evident in what is often called the “Social History” section of the PMHF. Nearly all the PMHFs I analyzed include some version of a Social History section, which typically asks questions about the patient-user’s exercise, smoking, alcohol intake, and drug use. Additional questions about social history will depend on the specific medical practice. One form from a sports medicine facility asks about caffeine use, whether the patient has tattoos and body piercings, and if they use birth

SOCIAL HISTORY				
Complete the following:		If Yes:	Complete the following:	
Do you use Alcohol?	☐ No ☐ Yes	drinks per	Educational Level	
Do you use caffeine?	☐ No ☐ Yes	__ drinks per __	Marital Status (circle)	Single Married Separated Divorced Widowed
Drug Use at any time?	☐ No ☐ Yes	Describe:	Exercise Habits	
IV Drug Use at any time?	☐ No ☐ Yes	Describe:	Do you always wear Seatbelts?	☐ No ☐ Yes
Do you use sun protection (sunscreen)?	☐ No ☐ Yes	Do you have Body piercings?		☐ No ☐ Yes
Do you have Tattoos?	☐ No ☐ Yes	Birth Control Methods		
Are you Sexually active?	☐ No ☐ Yes	Language spoken		
Religion			Occupation	

Figure 10. A social history section.

control and sun protection (Figure 10). Separating social history from family history brings attention to the patient's day-to-day behaviors and lifestyles that affect health and wellness. In this way, the form captures factors beyond genetic causation of health and risk, yet it still suggests individual responsibility of health and wellness rather than a distributed one.

The wider social context that frames health as an individual responsibility presents the social history section as a catalog of personal failures or achievements. A patient doesn't choose to have a family member with high cholesterol, but through the logics of individual responsibility, they choose to smoke, which is then framed as a personal failure. Alternatively, through logics of individual responsibility, patients who indicate they practice so-called desirable health habits are framed as personal successes. In either scenario, by simply capturing information about the patient-user, such as if they exercise, the PMHF circulates logics of individual responsibility and ignores wider social, political, and geocultural factors that affect what is possible for the patient and that could contribute to their own critical awareness of their circumstances.

Consequently, patient-users may not be as forthcoming in the social history section or might skip this section entirely because of the stigma that often sticks to social behaviors. In fact, the most common topics that patients tend to lie about are those that are typically addressed in the social history section: mental health, exercise, alcohol consumption, diet, drug use, sexual activity, and smoking (USARX, n.d.). Some patients worry about the stigma associated with socially undesirable behaviors, such as smoking and drug use, and might avoid answering questions out of fear of embarrassment or judgment (Krumpal, 2013; McDaniel et al., 1995). Others are afraid that their responses will be recorded in electronic medical records and shared with employers or they will lose health insurance or be unable to acquire health insurance in the future because of their so-called social behaviors, such as if they smoke (Reddy, 2013).¹⁰

Unlike other sections of the PMHF, in the forms I surveyed, the social history section is the only one that might be labeled as optional. This labeling might be because of the sensitive nature of the questions. Patient-users might interpret this optional status to mean that the social history information is less important than information in other sections because it is about the patient's social life instead of medical history. On one hand, marking the social history section as optional reflects clinical sensitivity by recognizing some patients might not want to reveal such information about themselves. On the other hand, patient-users might not want to reveal such information about themselves because the behaviors listed have historically been cast as potentially shameful personal failures or they feel these questions are beyond the scope of what should be addressed in the clinical encounter.

The PMHF across the information infrastructure

The logics of the PMHF are bolstered by other uses in its infrastructure that locate health, illness, and risk within the body and as an individual responsibility. In what follows, I track the different uses of

the PMHF across contexts, which include military enlistment, life insurance applications, and immigration processes. Through these republications, the forms continue to advance the logics of individual responsibility and are used as a way to assess to what extent the individual's body becomes a (mission, financial, legal, public health) risk.

A version of the PMHF is used for military enlistment through forms DD 2807-1 and DD 2807-02. The questions provide information on the candidate's medical background to determine "acceptability of applicants for military service" (United States Military Entrance Processing Command, 2018, p. 1). The questions are consistent with those asked in other PMHFs, but the uses of the information gathered is quite different. Positive responses do not necessarily disqualify a candidate but are reviewed by the Medical Evaluation Board "to determine medical fitness of a current member and if separation is warranted" (United States Military Entrance Processing Command, 2018, p. 1). It is assumed that any diagnoses or care will already have been addressed by other physicians. The form makes clear the information is not intended to help the candidate's health, and in signing the form, the applicant understands that "the MEPS [Military Entrance Processing Station] medical staff are not my healthcare providers . . . any concerns that I have about my health and healthcare are my responsibility to address with my personal healthcare provider(s)" (United States Military Entrance Processing Command, 2021). Regardless of the causes of illness or symptoms, the form locates the responsibility for health and risk within the individual candidate's body by way of possible disqualification from military service.

Similarly, the form can be employed to assess whether the applicant is a good investment for insurance or employment. Life insurance companies, for instance, often use a version of the PMHF, called a statement of health or medical evidence of insurability, to determine whether an applicant qualifies for coverage. Since the mid-1800s, "the life insurance industry had been interested in using medical history taking and physical diagnosis to assess the financial risk posed by life insurance applicants" (Han, 1997, p. 911). Forms might ask "a series of questions about your overall health – such as if you're a smoker or if you've ever been treated for a medical condition, like cancer or high blood pressure" (MetLife, 2023). Occasionally, these forms are accompanied by a physician's statement or medical exam. Regardless of the complex causes of health, illness, and risk, logics of individual responsibility are evident in how individuals can be denied coverage or required to pay an additional premium.

In other contexts, the patient's medical history assesses not their individual risks of disease but how the individual might be a risk to others. Individuals seeking residency in the United States complete USCIS Form I-693: Report of Immigration Medical Examination and Vaccination Record. Applicants use the form "to establish that [they] are not inadmissible to the United States based on public health grounds" (United States Citizenship and Immigration Services, 2023). This form differs from others in that a "civil surgeon" must conduct a physical assessment and complete the majority of the form on behalf of the applicant.¹¹ The form asks about test results, diseases identified during the exam, assessment of physical or mental disorders, addictions, and vaccine records. The information is then used by the Department of Homeland Security to determine immigration eligibility and to ensure that "aliens entering the United States do not pose a threat to the public health of this country" (Centers for Disease Control and Prevention, 2019).

The uses of these different forms leak into each other across time and contexts and, consequently, reproduce each other and the logics of individual responsibility. In fact, when presenting the PMHF in 1923 in the *JAMA*, Emerson refers to the Army's use of the PMHF and physical exam in its annual assessment of officers. He also refers to the data provided by life insurance companies. When detailing the physical exam process for preventive care, Emerson (1923) identifies the measurements physicians should take in the annual preventive health screening. Although he does not list what counts as normal measurements, he recognizes that life insurance companies have published tables of "normal" limits for height, weight, temperature, pulse, blood pressure, and blood sugar, presumably so physicians can refer to these charts on their own. These measurements inform the final part of the preventive health visit, which is to offer "advice in matters needing change in habits or conduct of life" (p. 1379).

The PMHFs that emerged throughout the twentieth century were variations of the forms that came before them. For instance, in 1979, Pecoraro et al. offers a PMHF that was tested for validity and reliability. To build the form, the research team “reviewed a variety of health history questionnaires in use by other institutions, clinics, and practitioners” and looked at forms reviewed by others (p. 232). They modified questions to suit their specific needs, but the scope of the form was similar to the CMI that came before and the PMHF Emerson offered in 1923, which were also informed by the forms used in military and life insurance contexts. In this way, the logics of responsibility of health, illness, and risk are reproduced in the information infrastructure.

Conclusion

Forms such as the PMHF inform, teach, and persuade users to adopt certain logics and values through enthymematic reasoning, yet, as so-called mundane texts, these rhetorical aspects of technical and medical forms are largely ignored. Forms have the potential to provide patient-users information about context and why certain data are being collected and how the data will be used. Indeed, as this analysis shows, the PMHF does much more than transmit health information from the patient-user to the medical provider; it also participates in larger rhetorical ecosystems that structure patient-users’ knowledges of and perspectives on medical topics.

When texts that are not typically seen as patient education materials are recast as performing patient education, we can identify opportunities to make these texts more informative, comprehensive, and just. The PMHF is in a position not only to collect health history information from patients but also to inform patients about medical subjects and complex factors of risk. Gathering health history does not need to, and should not, separate the patient-user from their “web of relationships that confer sickness or health” (Marya & Patel, 2021, p. 331). By reimagining mundane texts like the PMHF that interface with patients as performing patient education, medical communicators are potentially able to contribute to patient-users’ deeper understandings of health and risk by bringing their attention to the many factors beyond genetics and individual choice that influence health. For instance, the PMHF can add context that explains different causative and correlative relationships between family health and individual risk, all while creating a more inclusive form that accounts for diverse family structures. This context can also serve to reframe health and risk from an individual achievement or failure to a distributed one by accounting for the larger, intersecting systems that have a correlative effect on health. A family history of diabetes, for instance, can be set against the context of intersecting systems of food deserts, racism, and land degradation, which shows correlative (i.e., distributed responsibility) rather than causative (i.e., individual responsibility) factors of health. The premises of the requested information, when clearly stated, can contribute to patient-users’ critical awareness of their own situations.

A reimagined PMHF that centers the patient also provides information about multifactorial contributors to health, illness, and risk and decenters logics of individual responsibility. For instance, asking patient-users to list the zip codes where they have lived and how long they have lived there – and explaining why this information is being requested – can highlight the relationships between environment, health, and risk and address the potential health implications of increased human mobility. Importantly, the form can provide information to the patient-user on environmental factors of health that are common in those geographical areas. Such a shift more accurately reflects the reality of multiple factors that affect health and risk and is necessary as environmental, political, social, and geocultural factors continue to influence public health and individual risk in significant ways. Of course, other factors such as space, design, and medium limit what the form is able to do. Therefore, reimagining and redesigning the PMHF (and other forms) can make it dynamic and responsive to provide real-time explanations and context as patient-users complete the form. Such changes could also make the form more accessible for different users.

Through logics of individual responsibility, health and risk are framed as a personal (or genetic) success or failure rather than a social one. If reimagined as performing patient

education, the PMHF could provide the information and context for individuals to have a better sense of the individual, genetic, environmental, geocultural, and political factors that affect health and risk and to make space for the patient-user to interrogate their own circumstances. For instance, instead of asking if the patient exercises, it might ask if the patient has resources they need in order to exercise, if they have a safe place to exercise, or if they are satisfied with their exercise regimen. The form can also preface these questions with an acknowledgment that exercise looks different for different bodies and that larger political, economic, and social systems do not provide equal, affordable access to exercise spaces. A shift in framing the questions can reorient the patient-user to consider health not as a personal failing or success but as a social one. Such a shift is necessary to recast health disparities not as the result of an individual's race or disability, for instance, but of racism and ableism.

Drawing attention to the multiple factors of health and risk could have the added benefit of expanding social and political discussions about who is responsible for health and illness. In fact, the PMHF is in a position to provide the patient-user with critical context and information about how health and illness generally are driven by numerous, complex, intersecting systems that play out on their individual bodies. In Western discourse, issues of health, illness, and risk are typically located within the individual – that is, a person's genetics and individual choices are responsible for health and illness – which obfuscates other institutions' role in public health. Instead, distributed responsibility would bring attention to the influence and interconnectedness of local, national, and global industries on public health and individual risk, such as the role industries play in air and water pollution. Of course, revising the PMHF will not resolve all threats to public health or provide all necessary information to patient-users. Instead, it can be one of many ways a reimagined information infrastructure can amplify patient-users' critical engagement with health, illness, and risk. Informing patients of the multiple factors of health and risk through many different touchpoints is needed to help address health disparities between multiple publics.

Notes

1. For a more extended discussion of ideology and genre, refer to Coe et al. (2002).
2. A number of tools exist to help make patient education materials more accessible and usable, such as the Toolkit for Making Written Material Clear and Effective and The Patient Education Materials Assessment Tool.
3. Medical students are often taught the mnemonic OLD CARTS to gather the history of the present illness: Onset, Location/radiation, Duration, Character, Aggravating factors, Relieving factors, Timing, and Severity.
4. Although my analysis focuses on the PMHF as used in the United States, I cite medical histories of England here because American physicians often trained in England and Europe.
5. By one estimate, up to 80% of medical conditions can be diagnosed solely through a thorough medical history (Berdahl et al., 2022).
6. Emerson (1923) claims that a complete health exam should take 45 minutes to an hour.
7. If this movement continues to gain traction, I would not be surprised to encounter questions about firearm ownership and/or gun safety on PMHFs in the future.
8. Medical visits are already compressed for time, with the average visit with a primary care physician lasting about 18 minutes (Neprash et al., 2021).
9. This issue is also prevalent during pandemics, as Scott (2003) has noted. During the COVID-19 pandemic, patients visiting medical facilities would often need to answer yes or no on if they have been exposed to the COVID-19 virus (SARS-CoV-2).
10. The Patient Protection and Affordable Care Act (ACA) forbids health insurance companies from denying health insurance for individuals with health problems or preexisting conditions.
11. The legal consequences of these forms are also different depending on their context. In medical contexts, the patient-user has no legal responsibility for the form's accuracy. In other contexts, especially when the form is used by the federal government for military enlistment, immigration, and employment, the form typically includes a penalties statement. From Form I-693: "If you (the applicant or the civil surgeon) knowingly and willfully falsify or conceal a material fact or submit a false document with the Form I-693, we will deny your Form I-485 and may deny any other immigration benefit. In addition, you will face severe penalties provided by law and may be subject to criminal prosecution." Similar statements are included on the DD 2807-2.

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